
AFTER THE DEMO • FOR YOUR REVIEW

What you saw, and what we'd recommend.

A short, honest record of the demo — what it did on your own files, what it costs your staff in real terms, what happens when it doesn't work perfectly, and the one small thing we'd actually have you do first.

PREPARED FOR — TYPE A PRACTICE

Practices with no digital system today.

Nothing digital yet. A back room of paper. No view of the practice.

Written specifically for a practice in your situation.
The first step we recommend is the same for every practice — and it is deliberately the smallest one.

READ THIS
BEFORE YOU DECIDE.
IT IS DELIBERATELY SHORT.

You run on paper. In the demo, we took a stack of your own files — the disorganised ones, the handwritten ones — and showed you what comes out the other side: structured, searchable patient records, with the original scan one click away.

— Your own files, read automatically

Not a prepared sample. Your handwriting, your forms, your medical-aid stamps — extracted into structured fields while you watched.

— A question you can't answer today, answered

We asked the system a plain-English question across the records — the kind of question that today means an afternoon in the back room — and it returned the answer with the source page attached.

— Nothing on your desk changed

No new screen to learn during a consult. The work happens around you, not in front of you.

02 WHAT THIS ACTUALLY COSTS YOU IN STAFF TIME

The honest answer to the question you are really asking — “my reception is already drowning, what does this add to her day?”

<i>Per document</i>	30–60 seconds. The system extracts; your staff reviews what it found, corrects anything wrong, and approves. They know your handwriting; the system handles most of it; they catch the rest.
<i>Week one</i>	Scanning happens into the same folder your reception already scans into. The only new habit is the validation queue — a list of documents waiting for a 30-second check. That is the entire workflow change.
<i>Month three</i>	The queue is a background habit, not an event. The structured archive is large enough that the search and analytics start answering real questions. Nothing about the consult has changed.
<i>What does not change</i>	How you consult. What you write. Your existing system, if you have one. We are a layer that adds capability, not a system that takes work away from one that already works.

Demos show the happy path. This is the part that matters more: what the system does when a scan is bad, the handwriting is unreadable, or the extraction is unsure. We would rather you read this now than discover it later.

Low-confidence extraction

Every extracted field carries a confidence score. Low-confidence fields are flagged for the validator, not silently accepted. The human sees exactly what the system was unsure about.

An unreadable document

If a scan is too poor to extract, it does not get guessed at. It is held in the queue, flagged, and a person decides — rescan, enter by hand, or set aside. The system never invents data to fill a gap.

The wrong-patient safeguard

Nothing enters a patient's record until a person confirms it is the right patient. The match is shown; the human says yes or no. Wrong-patient errors are caught at that gate — before they happen, not discovered months later.

Every action is logged

Every approval, correction and reversal is recorded — who did it, when. If a medical aid audits the practice, or you need to show the basis of a record, the history is one query away.

04 WHY A SOUTH AFRICAN PRACTICE CHOOSES THIS

BUILT HERE, NOT BOLTED ON

Medical-aid schemes, HPCSA prescription rules, NAPPI codes, the SA edition of ICD-10, local payments — native to the system, not adapted to it after the fact.

IT WORKS WITH WHAT YOU ALREADY HAVE

Export and ingest via FHIR, CSV or API. You do not rip out a system that works to get the parts you don't have.

DIGITISATION AS A SUBSCRIPTION, NOT A PROJECT

The reason practices stay on paper is the six-figure project quote nobody signs. A predictable monthly page allowance instead — payable, cancellable, no capital sign-off.

EACH PRACTICE'S DATA IS ISOLATED

Multi-tenant by design. What happens at one practice never crosses to another.

The same first step regardless of how your practice is set up today — because every practice, paper or EHR or fully wired, has the same one gap that digitisation closes: a back room of legacy files that is dead weight until it is structured data.

Digitisation only. Nothing else, to start.

Not a platform. Not analytics. Not a bundle. The single thing we would put in your practice first is the digitisation of your legacy archive — because it is the one thing every practice needs, it is complete and useful entirely on its own, and it changes nothing about how you already work.

A real, complete product — not a foot in the door

Digitisation is not a teaser for something else. A practice that only ever digitises, and never buys another module, has still solved a real problem permanently. We mean that; the rest of this document is written so you can hold us to it.

Priced as a subscription, not a project

A fixed monthly page allowance, payable and cancellable, no six-figure project quote. We work through the archive at that pace. It takes time — and the predictability is the point, for you and for us.

Everything else this company sells is downstream of this one step — and it is downstream for a concrete reason, not a commercial one. See the next section.

This is the single most important thing in this document, so we are going to state it as the fact it is, not as a sales line. Analytics cannot run on a filing cabinet. The platform's value does not compound without structured history. Every capability beyond digitisation — the view of your practice, the cohorts, the search, anything further — needs the structured data that digitisation, and only digitisation, produces. That is not a reason to buy more. It is the reason the order is fixed: the archive becomes a data asset first, and whatever you choose to do with that asset comes later, on your timing. A vendor that sold you the downstream modules before the data existed would be selling you something that cannot work yet.

This section is not part of what you are being asked to decide now. It is here so you can see where digitising your archive naturally leads — because each step below becomes possible *only because* the digitisation produced structured data first. None of it works on a filing cabinet. All of it is a later conversation, on your timing, not ours.

Once the archive is structured — the analytics view

The moment a meaningful slice of your records is structured data, the questions you cannot answer today become answerable: which chronic patients are slipping, where revenue is leaking, who is overdue. This is the most natural next step for a practice in your situation, and it requires nothing but the data digitisation is already producing.

Further out — the full practice platform

If and when you decide to come off paper entirely — registration, encounter, prescription, invoice on one system — the structured archive means you are not starting from zero; your history is already in. This is a significant decision and the right time to make it is after digitisation has earned your trust, not on the strength of a demo.

The order matters and it is not negotiable: digitisation first, because everything else feeds on what it produces. A practice in your situation that tried to start with the platform would be migrating onto a system with an empty back room. We would rather build the data asset first and let the rest follow it.

The same discipline we apply to the product, applied to this document. These are real and they are not in scope of what you would be buying.

FINAL PRICING

Set per practice on doctor count, page volume and integration scope. The pilot price is fixed and stated; the full quote is a conversation, not a number on a leave-behind.

CLINICAL DECISION SUPPORT

Our clinical-AI work is an invite-only beta, not a commercial product, with no medical-device approval. It is a design-partner conversation if you raise it — it is deliberately not part of what this document offers you.

ROADMAP ITEMS

Medical-aid claims auto-submission, multi-site management and native EHR integrations are planned, not promised here. If it is not in this document, treat it as not yet real.

Not the catalogue. Not your type's eventual configuration. One small, bounded, cancellable first step — the same one we'd recommend to any practice, because it is the one that earns everything after it.

THE SMALLEST FIRST STEP

One month. Your files. Digitisation only.

Not the whole archive. Not a year's commitment. Not a bundle.

Digitisation only: one month, 1,500 pages of your choosing — ideally the messiest ones, so you are testing the hard case. Your staff validates as they would in production; the output lands in whatever system you already run, or in ours if you have none. At the end of the month you have a real, structured slice of your own archive and an honest sense of the day-to-day. No setup fee, no lock-in, cancel any time. Everything else is a later conversation that this step earns — or doesn't.

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